

## RECORD OF TELEPHONE CONVERSATION

**Submission ID:**  
27034

**Office:**  
OVR

(b) (6)

**Title:** *Human Coronavirus mRNA Vaccine (SARS-CoV-2 Spike Protein; BNT162b2) in Lipid Nanoparticles (ALC-0315, ALC-0159, DSPC and Cholesterol) (Pfizer-BioNTech COVID-19 Vaccine)*

**Sponsor:** *BioNTech Manufacturing GmbH (in partnership with Pfizer Inc)*

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**Telecon Date/Time:** 23-NOV-2020 04:30 PM

**Initiated by FDA?:** Yes

**Telephone Number:** (210) 795-0506

**Email?:** No

**Author:**

(b) (6)

**Entered in BIRAMS:** 25-NOV-2020 01:18 PM

**Last Revised:** 25-NOV-2020 01:18 PM

**Purpose:** *1. EUA OTHER (UO) - F/U T-CON TO DISCUSS THE SAFETY POP TO BE INCLUDED IN THE 508 COMPLIANT SAFETY & EFFICACY TABLES AND DATASET IRS*  
*2. EUA ADVICE/INFORMATION REQUEST (UE)*

### FDA Participants:

(b) (6)

### Sponsor Participants:

William C. Gruber, MD

Senior Vice President, Vaccine Clinical Research and Development, Pfizer Inc.

Donna Boyce

Vice President, Global Regulatory Affairs, Vaccines, Pfizer Inc.

Luis Jodar, PhD

Vaccines Chief Medical and Scientific Affairs Officer, Pfizer Inc.

Kenneth Khoury, PhD

Head of Statistics and Modeling, Vaccine Clinical Research and Development, Pfizer Inc.

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FDA-CBER-2021-5683-1804334

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|                            |                                                                                     |
|----------------------------|-------------------------------------------------------------------------------------|
| John L. Perez, MD, MBA, MA | Vice President, Vaccine Clinical Research and Development, Pfizer Inc.              |
| Carmel Devlin              | Global Regulatory Portfolio Lead, Global Regulatory Affairs – Vaccines, Pfizer Inc. |
| Elisa Harkins              | Senior Director, Global Regulatory Affairs – Vaccines, Pfizer, Inc.                 |

**Amendments:** 0 - ORIGINAL SUBMISSION

### **Summary of Discussion:**

F/U T-CON TO DISCUSS THE SAFETY POP TO BE INCLUDED IN THE 508 COMPLIANT SAFETY & EFFICACY TABLES AND DATASET IR's - see EDR for full communication.

### **Background:**

On the morning of November 23, 2020, (b) (6) e-mailed Ms. Donna Boyce at Pfizer the below summary of their November 22, 2020, teleconference.

Hi Donna, I'm summarizing our phone call last night and follow-up emails for my records to share with my team. Please let me know if this aligns with your records:

- I inquired about the safety population that would be included in the 508-compliant safety and efficacy tables that we were expecting to receive from you today, using the table templates we provided to you last week, and explained why we thought it would be best for those tables to include the all enrolled population of >43,000 subjects with data cutoff of November 14 rather than the smaller safety population of nearly 38,000 subjects defined by a median follow-up of 2 months.
- You will check with your team today about providing the tables referenced above using the larger all enrolled safety population. I stated that if doing so would cause a significant delay, we would need to go with the smaller safety population so as not to impact our review.
- You will also inquire with your team about providing additional safety tables (e.g., demographic subgroup analyses) that were included in the November 20 EUA request for the smaller nearly 38,000 subject safety population but not for the larger all enrolled population.
- Because the larger all enrolled safety population does not have a median follow-up of 2 months, our intent would be to explain that the follow-up for the all enrolled population is adequate because it includes the nearly 38,000 subject population that meets the expectations in our guidance for both efficacy and median duration of follow-up, and we are presenting data for the overall population in the interest of more comprehensive safety and a more precise estimate of efficacy.

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- With regard to the nearly 38,000 subject population with median follow-up of at least 2 months, I asked if the primary efficacy analysis could be run on this population as a post-hoc analysis. To clarify further now, this post-hoc analysis would be for our internal use only, to verify the underlined text in the bullet above. You will inquire with your team about providing this analysis.

### **Summary of Discussion:**

Teleconference regarding the safety population that would be included in the 508-compliant safety and efficacy tables. See details below.

### **Teleconference Summary:**

CBER asked about the population that would be included in the 508-compliant safety and efficacy tables that we were expecting to receive from Pfizer today (November 23, 2020) using the table templates we provided to them last week, and inquired about including in those tables the all-enrolled population of >43,000 subjects with data cutoff of November 14th rather than the smaller safety population of nearly 38,000 subjects defined by a median follow-up of 2 months, given that the EUA submission includes safety data and some safety analyses for all enrolled subjects. Pfizer explained that the initial submission (dated November 20, 2020) included three types of safety analyses – (1) core population of 38,000 subjects that have 2 or more months of median follow-up; (2) many subgroup analyses, for sex, race and ethnicity, that have 2 or more months of median follow-up, and (3) all AEs for all of the 43,00 subjects from the November 14th cut off; this safety population does not have a median follow-up of 2 months. The safety population of all AEs/subjects is missing the sex, race and ethnicity subgroup analyses. Pfizer stated that safety data on the additional subjects contributing to the all-enrolled population of >43,000 would require further cleaning and quality control prior to compiling into tables for submission, and due to the Thanksgiving holiday, it may take more than a week to submit the additional information. As the safety and efficacy tables are linked to both VRBPAC briefing document and the EUA Full Prescribing Information, Pfizer stated that the timeline to submit all these documents would also be delayed.

CBER concluded that given the timelines explained by Pfizer, the EUA review, VRBPAC presentation, Prescribing Information would focus primarily on the analyses already submitted for the core population of nearly 38,000 subjects, plus notable findings (e.g., serious AEs) from review of the additional data and available analyses for the all-enrolled population of >43,000 subjects. Pfizer suggested that they will also present the same data that is in the submission (from 38,000 subjects) at the VRBPAC meeting and will keep the cleaned data from the larger population (43,000 subjects) in back-up slides. CBER agreed.

Pfizer stated that they have the safety tables that are ready to be submitted today. However, they won't be able to submit the efficacy tables, VRBPAC briefing document and the EUA Full Prescribing Information until the end of Wednesday, November 25, 2020. CBER acknowledged.

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CBER asked if solicited AEs are also included in the larger (43,000 subjects) population. Pfizer replied that they didn't collect solicited AEs for all subjects; solicited AEs were collected from only 8,000 subjects (6,000 U.S. subjects and 2,000 subjects from other countries). CBER acknowledged.

Pfizer indicated that they were not intending to submit efficacy subgroup analyses for CBER-requested age groups, e.g., for participants 16-17 years of age and for participants 75 years of age and older. As the number of participants in these age groups is considerably lower, the subgroup analyses will not be meaningful. CBER explained that the request for these subgroup analyses was to ensure adequate understanding of the data in case questions arose at the VRBPAC. Pfizer acknowledged.

Pfizer also indicated that they don't have information to complete the subgroup analyses tables regarding "At high risk – Yes/No" item, and they asked for clarification regarding definition of "high risk" population. CBER stated that it is up to Pfizer to categorize the subjects within the data they have collected (e.g., comorbidity, obesity) and to provide subgroup analyses for individual comorbidities also, if possible. CBER requested that Pfizer have a demographic on subjects with obesity and Pfizer said they planned to and they have good data, about 35% of the population. CBER acknowledged.

CBER asked for details regarding how denominators were calculated for C4591001 IA Efficacy and FA Efficacy datasets, the total population and flags, and the rationale for excluding HIV+ve subjects. Pfizer stated that the efficacy population contains 43,325 subjects (as denominator) from the November 14th cutoff. They indicated that they don't have the appropriate personnel on the call to discuss datasets and requested if the issue can be discussed in a separate technical meeting. CBER stated that we will arrange for a technical meeting this week and will send a list of all issues for discussion. Pfizer agreed.

**Post-teleconference note:** A technical working group telecon between some of the review team members (clinical, stats, dataset integrity and regulatory) and Pfizer subject matter experts was scheduled to occur on Wednesday, November 25, 2020, from 10:30-12:00 discuss data IRs.

Telecon ended.

Signature: \_\_\_\_\_ Mail Code: \_\_\_\_\_